

Title

Implementation outcomes and organizational factors in scaling community health worker-mediated hypertension programs across rural facilities

Abstract

Background: Understanding implementation success requires assessment of organizational readiness and health system integration factors. This secondary analysis examines implementation outcomes from the SEARCH Sapphire hypertension trial. **Methods:** Implementation data were collected from 10 participating healthcare facilities (5 intervention, 5 control) in Kenya and Uganda. Data sources included provider interviews (n=15), patient interviews (n=40), and health facility records. Analysis examined workflow integration, technology adoption, staff training adequacy, and perceived sustainability. **Results:** Intervention facilities successfully integrated CHW-remote consultations into routine workflows; mean integration time was 3.2 weeks post-training. Provider workload was comparable between arms. Training completion was 92% at intervention sites. Perceived sustainability was higher at facilities with designated CHW coordinators and reliable communication infrastructure. Key barriers included intermittent telephone connectivity (40% of facilities), lack of standardized treatment protocols (35%), and limited incentive structures for CHWs (50%). Facilitators included strong facility leadership support and prior experience with community-based programs. **Conclusions:** CHW-mediated remote care models require attention to organizational infrastructure and health system alignment for successful and sustainable implementation.